MedTech & Healthcare Services Sector

Impact of DX and AI adoption in healthcare: Initiatives that will lead to future market share gains are underway behind the scenes

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Digital transformation (DX) and AI use seem to have recently advanced under the radar at healthcare facilities, and we summarize near-term implications. Within our coverage, we believe this could contribute to longer-term market share growth for monitors and other products in Nihon Kohden's digital healthcare solutions (DHS) segment, although the scale of this business is still small. In the healthcare services sector, M3 and JMDC are moving forward in using AI in various businesses, and while we expect longer-term contributions to enhancing competitive advantages and raising the profitability of new services, we focus on higher AI-related investment in the short term (increases in engineer hiring, AI system usage fees, and capex).

- **Implications for Nihon Kohden (6849):** Nihon Kohden's medium-term business plan outlines a strategy to expand DHS sales by focusing on alarm solutions for medical staff providing inpatient care and on systems that facilitate inpatient monitoring and management. We expect the adoption of DX and AI in healthcare services to increase from July 2026 as Japanese hospitals use government subsidies to deploy ICT equipment and software to enhance the operational efficiency of healthcare facilities and improve workplace environments (application submissions are scheduled to open in June 2026). At Nihon Kohden, although the scale of sales for individual DHS products is not large, we believe the gradual recognition of its DHS products at US hospitals while it builds a deployment track record in Japan could contribute to raising its US market share for patient monitors (installed basis). In fact, new DHS products contributed to monitor sales growth in the US in FY2024, and with this potentially leading to large projects, we believe it is important from a longer-term perspective. Following the end of Covid-19-related special demand, Nihon Kohden had especially weak earnings and low valuations from FY2024 due to worsening market conditions in Japan and some overseas markets and to higher wages. However, it has been discontinuing low-profit businesses and implementing structural reforms since FY2025. In addition, Nihon Kohden's monitor sales in Japan resumed double-digit (%) growth in YoY in 4Q FY2025, and as it seems that capex at Japanese hospitals is already showing signs of recovery, we expect the share price to rise toward 2Q FY2026 (Nihon Kohden said improved market conditions in Japan should fully contribute to earnings from 2H FY2026).
- **Changes needed at medical device manufacturers:** Healthcare facilities have been moving quickly to experiment with DX and AI since the broader launch of generative AI services, and recent examples of DX and AI use at hospitals and clinics were presented at the Eucalia Friendship Meeting on July 4 (we summarize the takeaways from page 3). Japanese hospitals have been able to apply for subsidies under a government program to enhance operational efficiency and improve workplace environments since July 2026. The program offers subsidies up to ¥80 million per hospital to support the deployment of ICT equipment and software, including for communications equipment, AI

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medical consultation, generative AI document creation, conveyance robots, monitoring devices, and network infrastructure. However, with medical institutions introducing advanced computer and systems infrastructure for using AI (see our report here for initiatives by NVIDIA and others to provide healthcare AI infrastructure), it seems healthcare facilities are investing more than we expected in DX and AI. We believe the effective use of limited funds is important for healthcare facilities. Before Covid-19, there were a number of challenges in applying DX and AI to healthcare, including divergence with the actual needs of healthcare facilities, and issues related to precision, safety, ethics, and operations (Figure 15). Recently, however, the number of approved medical devices that use DX and AI has increased, and while we do not see problems with the products themselves, it seems to us that competing medical device manufacturers have released a growing number of similar AI products. In this environment, we believe a key issue now is whether products have the competitive advantages and unique features needed to make them the preferred choice of healthcare facilities. For example, Fukuda Denshi's electrocardiographs with AI analysis capabilities can perform AI diagnosis similar to competing products, but the ability to use this system with lower-performance computers is a unique advantage compared with competing products. With a recent focus on the risk of higher fixed costs for DX and AI deployment at healthcare facilities, we think medical device manufacturers, when developing products that use DX and AI, need to consider not only improving diagnostic and treatment capabilities, but also the capex burden for healthcare facilities.

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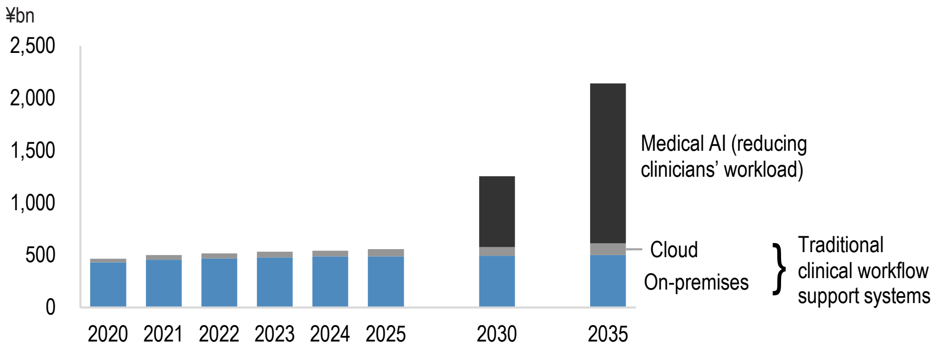
Rapidly accelerating adoption of DX and AI on the medical front

Takeaways from meeting on DX and AI use on the medical front

The Eucalia Friendship Meeting on July 4 showcased many recent examples of the use of digital technologies and AI on the medical front from the perspective that hospital and clinic management should essentially maximize the time medical professionals spend with patients, leading to wide-ranging discussions. We summarize the main points below.

Figure 1: Clinical workflow support systems Japan market size



Source: Medley estimates, J.P. Morgan
Note: Historical data are Medley estimates.

(1) AI significantly reduces documentation time in medical practices

In one example at a hospital, use of AI enabled the generation of electronic medical records in less than sixty seconds by capturing the conversation between the patient and the medical professional through voice recognition and generating the text in the SOAP format (with subjective information, objective information, assessment, and plan of care). In addition, the use of Claude allowed medical professionals to compose routine emails and informational materials 1.8 times faster.

(2) Some hospitals use proprietary data

The meeting showcased a hospital that created a dashboard for analyzing data in response to the medical fee revision in June 2026. Electronic medical records in particular are connected to various departmental systems, so the hospital plans to collect data from these systems and use them to improve operations and business efficiency.

(3) Automation of medical paperwork not only reduces hospital business hours but also leads to retroactive billing for compensation

The future direction of medical administration was discussed. Hospitals and clinics have faced the issues of increased specialized work related to medical reimbursement claims and chronic labor shortages in recent years. The meeting highlighted an example of a hospital that reduced man-hours by 550 hours a month, the equivalent of 3.6 employees, by using robotic process automation (RPA) and automating medical administrative operations. This enabled all staff members to focus on reception and patient-facing work during the day rather than internal administrative tasks. By using RPA, the hospital also identified tens of millions of yen in medical claims that had previously been overlooked due to dependence on human skills, and was able to request retroactive reimbursements.

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(4) AI agents could replace some of hospitals' reception operations and specialist classifications, possibly causing hospital and clinic capex to rise for AI use

The meeting showcased an AI agent co-developed by Fujitsu and NVIDIA for healthcare providers that helps receptionists assign specialists to patients. The product enables AI to take over tasks such as patient intake and interviews by standardizing previous intake interviews and letting patients speak with an AI avatar on a screen when arriving at the hospital. AI can also take over the task of assigning patients to appropriate specialists, automating a series of intake tasks previously performed by medical professionals. Going forward, the companies plan to screen patients on their smartphones by conducting interviews tailored to patients ahead of time, proposing a specialist to see, and advising whether an online visit is an option. These initiatives aim to facilitate a hospital's first contact with a patient. Because hospitals need computers with a certain level of sophistication to safely handle confidential medical data internally and use generative AI, we believe demand for desktop AI supercomputers might increase.

(5) Prevalence of online medical visits

Online medical visits are currently the most widespread in China and India. In Japan, the majority of online visits are used at clinics, but hospitals often use doctor-to-doctor online service to allow cardiac surgeons and other physicians to collaborate with specialists at remote locations. The use of products such as Join, DeNA's app for communication between medical professionals, is increasing.

The key question is whether digital solutions and AI functionality will be recognized and valued, ultimately driving an increase in device market share

Nihon Kohden: DHS's business scale still small, but focus is on whether it contributes to higher monitor share in North America

Nihon Kohden's medium-term plan strategy is to expand digital health solutions (DHS) sales to hospitals, and we expect new products including alarm solutions and next-generation central monitors to contribute to earnings from FY2026. At Japanese hospitals in particular, we expect increased use of the Japanese government's supplementary budget intended to improve operational efficiency and workplace environment on the medical front through the use of information and communications technology (ICT) equipment from July 2026 onward, increasing the use of digital technologies in medicine. We also believe the medical fee increase at hospitals from June 2026 has improved their business environment, providing some reassurance. We thus think this could provide some tailwinds to DHS product sales of Nihon Kohden's patient-monitoring business. While the scale of the company's DHS sales is still small, we believe the profit margin is better than that of equipment products. As a future focal point, while Philips and GE HealthCare now hold high shares in the US market, we believe what is important is whether Nihon Kohden's DHS is well received by hospitals and contributes to an increase in its patient monitoring systems' US share (on the basis of equipment installed).

Figure 2: Nihon Kohden: Key New Products and Services for Improving Hospital Management Efficiency and Healthcare Economics

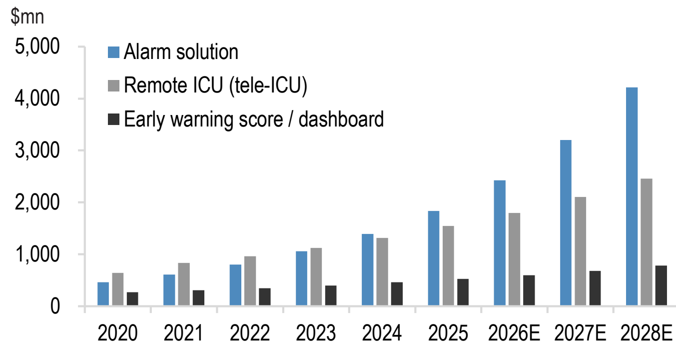
Period	Region	Business	Products / Services
FY3/25		Solutions	Patient Condition Dashboard Software (for general wards)
FY3/25		Solutions	Patient Condition Dashboard Software (for critical care wards)
FY3/26		Solutions	Integrated Dashboard for PFM Center
FY3/26 2Q	U.S.	Solutions	Alarm Solution "AlarmSense"
FY3/26 3Q	Japan	Patient Monitors	Transmitter ZS-730/720P
FY3/26 4Q	Japan	Solutions	On-site Alarm Analysis Software
FY3/26 4Q	Japan	Solutions	Admission/Discharge Workflow Support Software
FY3/27	Europe	Solutions	Alarm Solution "AlarmSense"
FY3/27	U.S.	Patient Monitors	Next-generation Central Monitor

Source: Company data, company estimates, J.P. Morgan
Note: Solutions business = DHS (Digital Health Solutions) + ITS (IT Solutions); PFM (Patient Flow Management) Center: collects and analyzes in-hospital data in real time, and bed utilization and the patient intake/acceptance status are centrally managed in an integrated manner.

Figure 3: Nihon Kohden: DHS+ITS products Japan sales

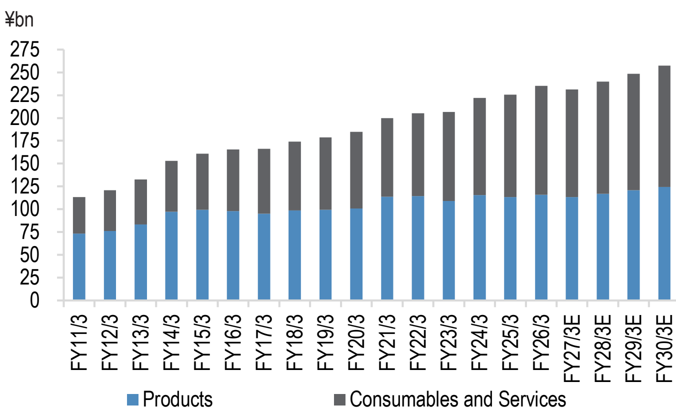
Source: Company data, J.P. Morgan

Figure 4: DHS products North America market size



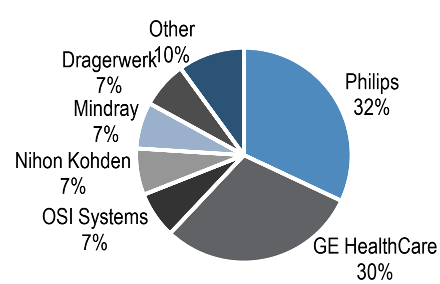
Source: Nihon Kohden, Nihon Kohden estimates (from 2026), J.P. Morgan

Figure 5: Nihon Kohden: Sales by type



Source: Company data, J.P. Morgan estimates

Figure 6: Patient monitors global market shares (2024)

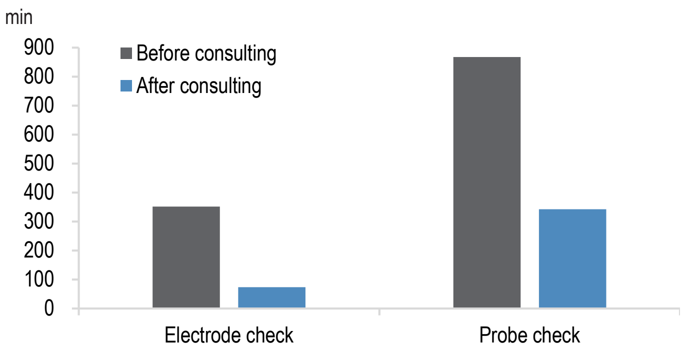


Source: Spherical Insights, Precedence Research, J.P. Morgan

Nihon Kohden: Solutions to reduce alarm fatigue among healthcare professionals at hospital wards

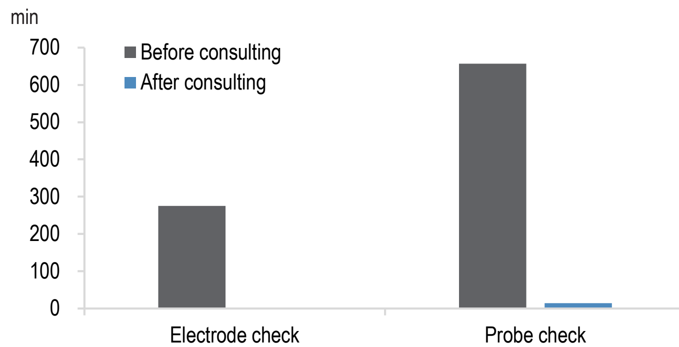
One of Nihon Kohden's strengths is its sensor technology for devices that measure patient biometrics such as blood oxygen saturation, exhaled carbon dioxide, pulse, and brainwaves. The company offers high-precision alarm management systems based on this information. When a hospitalized patient's alarm goes off now, a medical professional goes to the patient to verify the situation in person, but this has become an issue on the medical front, with some data suggesting that verification is unnecessary about 85% of the time (as of 2026, source: Nihon Kohden material). To solve this issue with alarms, Nihon Kohden provides alarm reports to the medical front (its service staff collects and analyzes alarm data offline and proposes operational improvement), launching an on-site alarm analysis software in FY2025. This software displays the occurrence and causes of alarms by patient in real time and on-site, making it easier to manage alarms on the medical front. On-site alarm analysis software QP-841N, launched in Japan in February 2026, is priced at ¥1.5 million, and the company expects to sell more than 100 sets by FY2028, mainly to Japanese healthcare providers. It plans to continue investing in R&D of high-precision alarm detection technology and provide full-scale consulting services for hospital ward alarm management (Figures 7-8).

Figure 7: Nihon Kohden: Alarm-state duration



Source: Company data (2026), J.P. Morgan

Figure 8: Nihon Kohden: Alarm-sound duration



Source: Company data (2026), J.P. Morgan

Fukuda Denshi: AI diagnostics raise detection rates of atrial fibrillation symptoms and contribute to prevention, can be used with non-high-end computers

Fukuda Denshi (Not Covered), which competes with Nihon Kohden, launched CardiMax 9 Ai (FCP-9900Ai system), an electrocardiograph with AI analysis function, in 2024. This system can estimate past atrial fibrillation, which was difficult to do with conventional electrocardiographs, and we believe it can contribute to early detection of atrial fibrillation risk and prevention of attacks. Fukuda's technology was featured in the July 2, 2026 edition of Nature (see here for the article). In addition, while several overseas competitors have also launched similar AI diagnostics products, we believe the strength of Fukuda's product lies in the use of algorithms that can be used not only on high-end computers that are suitable for AI use but also on computers with standard CPUs for general use. With an increase in fixed costs for AI use viewed as a risk on the medical front, we expect a stronger need for technologies like this that can reduce hospitals' capex burden for using AI.

Status of DX and AI adoption among medical device manufacturers

Use of data analysis and AI is advancing in diagnostic imaging

The use of AI is advancing in the diagnostic imaging field. It was recently reported that about 20% of early stage cancers were missed during past endoscopic examinations (source: AI Medical Service, 2026), and multiple AI diagnostic products have been approved. We see collecting endoscopic images is not easy for individual companies, and we think this has become a bottleneck in developing AI endoscopy products. Olympus also does not have an in-house system for collecting examination images, despite a global market share for gastrointestinal endoscopes in excess of 60% (as of 2026), and we believe this process requires cooperation with physicians and costs.

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Global market shares for gastrointestinal endoscopes have fluctuated in recent years (Fujifilm's market share has recently risen), and simple initial screening tests that can replace endoscopy have also increased. We think R&D for products that can reduce the burden for patients and improve endoscopy accuracy is important over the longer term.

Figure 9: Endoscopy AI products

Examination Method	Examination Site	Main AI Products
Upper GI Endoscopy	Oral Cavity Pharynx, Larynx Esophagus Stomach Duodenum	AI Medical Service (gastroAI model-G2) Fujifilm (CAD EYE) Olympus (esophagus only, CADU)
Capsule Endoscopy	Small Intestine	None
Lower GI Endoscopy	Colon Anus	Fujifilm (CAD EYE) Olympus (EndoBRAIN, CADDIE) AI Medical Service (gastroAI model-EIRL) NEC (WISE VISION)

Source: Company data, J.P. Morgan

Use of data analysis and AI is limited in treatment equipment and IVD testing, but is gradually advancing for new businesses and plants

Compared with diagnostic imaging, the use of data analysis and AI in medical devices for treatment and surgery seems limited at present. However, Terumo, which mainly handles cardiovascular products, plans to focus on the imaging business (IVUS imaging, OFDI imaging), and is pursuing R&D to use AI in this new business (Figure 10). Asahi Intecc, which similarly focuses on cardiovascular products, is experimenting with using data analysis and AI to enhance plant productivity and shorten document preparation work, including for regulatory approval (see our report here for initiatives at the Thailand factory).

Figure 10: DX & AI utilization in vascular imaging

Company	Modality	Overview of Key DX & AI Utilization
Terumo	IVUS, OFDI	Image fusion and analysis support using OPUSWAVE, which integrates IVUS and OFDI; currently under R&D leveraging AI (stent placement recommendations, calcified plaque assessment, endpoint evaluation).
Philips	IVUS	Automated measurements of vessel diameter, lumen, and lesions; PCI workflow support; AI-based image analysis.
Boston Scientific	IVUS, OFDI	AI-driven lesion analysis (Automated Lesion Assessment), automatic extraction of vessel contours, PCI support.
Abbott Laboratories	OCT (related to OFDI)	AI-based image analysis via Ultreon Software, calcium detection, stent size recommendations.
Conavi Medical	IVUS, OCT	Hybrid image analysis and AI-enabled image processing.
Evident Vascular	IVUS	AI-enabled IVUS platform.

Source: Company data, J.P. Morgan

In the area of IVD testing, which is a core business at Sysmex, although the number of testing devices equipped with AI capabilities is currently limited, the use of AI in laboratories is gradually expanding overall. For example, Roche and others are using AI to forecast consumables replacement, equipment breakdowns, calibration errors, and downtimes with the aim of improving laboratory capacity utilization and reducing maintenance costs. Companies are also working to support diagnostics, including quality control via lot abnormality detection for reagents and patient risk assessment via AI analysis of test results. In addition, as part of Sysmex's long-term vision, although we think it will take considerable time to contribute to earnings, it aims to use data analysis in the APAC region, including India, to increase equipment utilization rates and become the preferred brand of governments in various regions from the standpoint of medical cost performance (see our report here).

Figure 11: DX & AI utilization in IVD testing

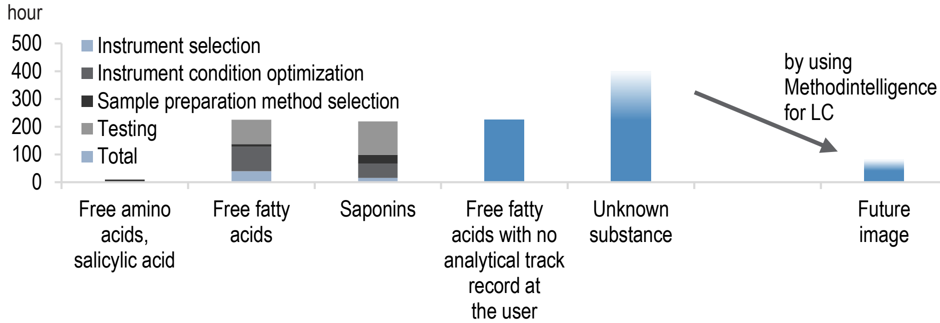
Company	Main Products	Overview
Sysmex	DI-60, UN Series	AI-powered blood cell image analysis system. AI-based blood cell classification, anomaly detection, cloud-based data analytics, and development of disease risk prediction models. Global deployment, achieving both testing accuracy and efficiency.
Roche Diagnostics	Integration with navify	AI-driven test data analysis and automation, abnormal value detection. Disease risk prediction and testing efficiency improvement leveraging large-scale data. Deployed in hospitals and testing centers globally.
Siemens Healthineers	Atellica Solution	AI-based automatic test result interpretation, QC anomaly detection, reagent consumption forecasting, specimen transport optimization, and lab-wide data integration and analysis. Automation and AI utilization for large-scale laboratories.
Abbott Laboratories	Alinity, AlinIQ	Data analysis, AI-driven testing process optimization, QC anomaly detection, predictive equipment maintenance, and real-time analysis of test data. Improving testing accuracy and laboratory operational efficiency.
Mindray	MC-80, LabXpert	AI-based blood cell classification, specimen processing and image analysis, automatic test data interpretation. Automated validation. Expanding adoption in emerging markets.

Source: Company data, J.P. Morgan

In the life sciences sector, leveraging AI can provide a competitive advantage as well

Shimadzu introduced several new software products at its September 2025 briefing on the analytical & measuring instruments (AMI) division (see pages 22–24 of the materials). Its new AI-analysis method creation software, Methodintelligence for LC, uses a system similar to generative AI, and when a user enters text about the substances and conditions they want to analyze in Japanese or English, it instantly proposes the optimal measurement conditions, and the best measurement conditions for the user's instruments based on data from past papers and Shimadzu's proprietary analytical data. The company's AI specialist team took several years to develop the software. We believe the entry barriers are high because development depends not only on data from published papers but also Shimadzu's accumulated analytical data. The new product has not yet been released, and the company envisions offering it under a monthly subscription model. The company is ahead of rivals in developing software that saves time and effort required for analysis (it uses AI to cover tasks like analysis method creation, automatic waveform processing and abnormal peak detection), and we believe it will wield a competitive edge going forward. Also, we get the impression Shimadzu's operations generally face a small risk of substitution by AI. We think that AI cannot directly replace measuring instruments, but that measuring instrument customers (such as pharmaceutical makers) have started to use AI for some R&D processes, so measuring instrument usage frequency may fall over the long term.

Figure 12: Example of the time required to evaluate analytical conditions in LCMS



Source: J.P. Morgan estimates based on Soichiro Okita et al., Tottori University, 2023

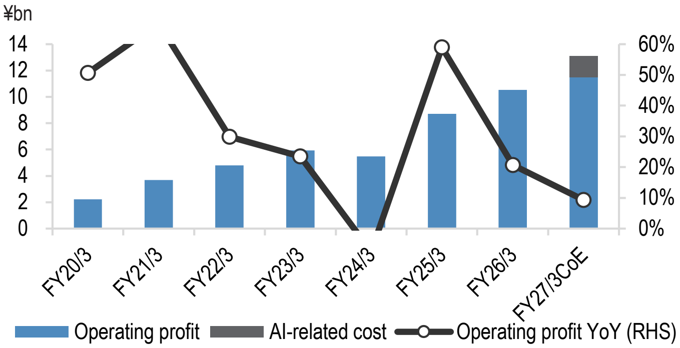
Impact of AI use in healthcare sector

Longer term, it may strengthen companies' competitive edge and improve the profitability of new services; near term, we will keep a close watch on rising costs

JMDC operates healthcare big data analysis as a core business, and this business has long had a strong connection with digital transformation (DX) and AI use as a whole. Regarding the impact of AI evolution on the company, management said at 4Q FY2025 results that, instead of replacing its consulting service and analysis service, AI has lowered the analysis hurdle and accelerated the hypothesis verification cycle, leading to faster discussions with client companies and proposal activities. It said the seeds of its projects are indeed growing, and new business opportunities combining data from companies in other industries are widening. In addition, given the high reliability required from healthcare data, JMDC said it does not expect such data to be replaced much by synthetic data. At the same time, it announced plans to accelerate strategic investment in AI in FY2026 given the changes in the industry and AI evolution. Its FY2026 guidance calls for just 9% YoY growth in operating profit versus 20% growth in sales. JMDC explained that FY2026 is the time to accelerate AI-related investment including human resources and infrastructure for medium-term business growth (nearly 50% of the strategic investment will be infrastructure investment for AI use and to increase AI engineers, while slightly more than 50% will be investment to increase data consultants). While the company wants to essentially finish hiring engineers and data consultants in FY2026, we are concerned about the possibility of this being delayed into FY2027.

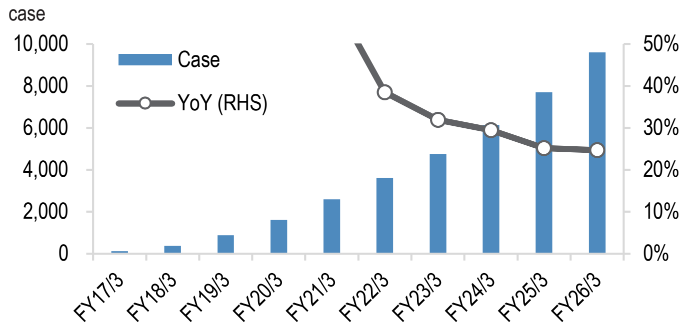
M3 showcased various products and services incorporating AI at 4Q FY2025 results. It said AI is increasingly replacing operations in its core pharma marketing support business, leading to a 20% productivity improvement versus FY2019, and also said it plans a significant productivity improvement going forward. It said AI has also made the targeting of physicians for marketing new drugs more efficient than before. However, we think that M3 seems to us to believe that AI products themselves could become commoditized over the long term. We believe its competitive advantage lies in its ability to obtain large amounts of proprietary healthcare industry data from its own platform. In particular, it has M3 DigiKar, the cloud-based electronic medical records app with the top market share in Japan (as of 2026), so we believe it is easy to obtain a wide range of the latest data. Since M3 estimates the number of patients and drug prescriptions using these proprietary data, we see little likelihood of its pharma marketing support business being replaced by other companies. However, in the near term, we see the risk of intensifying competition with rivals in the Japanese market for cloud-based electronic medical records including Medley. With the Japanese government targeting a 100% penetration rate for electronic medical records, we believe the environment for the cloud-based electronic medical records industry is changing. WEMEX (a PHC Holdings subsidiary), which originally held the top share in on-premise electronic medical records for Japanese clinics (as of 2022), is now growing hybrid medical records, which offer the advantages of both on-premise and cloud-based electronic medical records, as well as 100% cloud-based electronic medical records. We will watch the impact of increased industry competition on M3, including a one-off increase in SG&A expenses.

Figure 13: JMDC: Operating profit & AI-related investment



Source: Company data, J.P. Morgan

Figure 14: Cumulative number of medical institutions using M3 DigiKar



Source: Historical data are J.P. Morgan estimates based on company data

Figure 15: Cases of challenges in DX & AI utilization in healthcare before Covid-19

Company	Period	Case
IBM Watson for Oncology	2015~	Accuracy issues with cancer treatment AI: Deployed at globally renowned cancer research centers, but concerns were raised about the accuracy of treatment recommendations and discrepancies with actual clinical practice, leading to discontinuation of adoption.
Google Health	2016~	Difficulty in practical application of AI breast cancer diagnosis: Although the AI's diagnostic accuracy for breast cancer was high, operational deployment in actual clinical settings proved difficult, and parts of the project were suspended.
Google DeepMind Health	2016~	Patient data privacy issues: In an AI project with the Royal Free Hospital in the UK, the handling of patient data became problematic, and the organization received warnings from regulatory authorities.
Babylon Health	2017~	AI misdiagnosis issues: The AI chatbot produced misdiagnoses for certain symptoms. Cases arose where its scope of use within the UK's NHS was restricted.

Source: Company data, J.P. Morgan

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